

1. Administrative & Study Identification

Full Study Title: A Study of BMS-986484 Alone and Combination Therapy in Participants With Advanced Solid Tumors
Short Title/Acronym: BMS-986484 in Advanced Solid Tumors
Protocol Number: CA233-0000
NCT Number: NCT06544655
Posting ID: 304306983368310241
Trial Status: Recruiting (Currently recruiting participants)
Sponsor Name: Bristol-Myers Squibb
Investigational Product: BMS-986484 (administered alone or in combination with Nivolumab, Oxaliplatin, Capecitabine, Fluorouracil [ATC Code: L01BC02; Trade Names: Carac, Efudex, Adrucil, Fluoroplex, Tolak], and Calcium folinate)
Phase of Development: Phase 1/1b
Medical Monitor: Bristol-Myers Squibb Clinical Study Director

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To assess the safety and tolerability of BMS-986484 administered alone and in combination with nivolumab and/or other standard chemotherapy agents in participants with advanced/metastatic solid tumors.
Secondary Objectives: To evaluate the pharmacokinetics (Cmax, Tmax, AUC), immunogenicity (anti-drug antibodies), and preliminary antitumor activity by RECIST v1.1 (including objective response rate, disease control rate, and duration of response).

2.2 Background & Rationale To address the ongoing medical need for effective treatments in patients with locally advanced unresectable, metastatic, or recurrent solid tumors (including NSCLC, PDAC, G/GEJC, MSS CRC, and SCCHN). This First-in-Human study investigates the safety profile and potential synergistic efficacy of the novel agent BMS-986484 when used as a monotherapy and combined with established immunotherapies and chemotherapies.

3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional
Control Type: Single-arm (Dose escalation and expansion cohorts)
Blinding: Open-label
Randomization: N/A

3.2 Planned Enrollment & Duration Target \$N\$: Approximately 213 participants

Number of Sites: Global (Multiple locations including the United States, Japan, Australia, and Canada)
Estimated Study Start: 10-Oct-2024
Estimated Primary Completion: 14-Oct-2027

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Locally advanced unresectable, metastatic, or recurrent malignant tumors including non-small cell lung cancer (NSCLC), pancreatic ductal adenocarcinoma (PDAC), gastric/gastroesophageal junction adenocarcinoma (G/GEJC), microsatellite stable colorectal cancer (MSS CRC), and squamous cell carcinoma of the head and neck (SCCHN). 2. Must have measurable disease by response evaluation criteria in solid tumors version 1.1 (RECIST v1.1). 3. Must have an Eastern Cooperative Oncology Group (ECOG) Performance Status of 0 or 1.

4.2 Exclusion Criteria 1. History of or active interstitial lung disease or pulmonary fibrosis. 2. Active, known, or suspected autoimmune disease. 3. Serious uncontrolled medical disorders, or new onset, non-catheter-associated venous thromboembolism within the past 6 months. 4. Other protocol-defined Inclusion/Exclusion criteria apply.

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: Ascending dose levels of BMS-986484 evaluated in cohorts, administered alongside combination therapies depending on the designated study arm.
Route of Administration: Intravenous (IV)
Duration of Treatment: Until disease progression, unacceptable toxicity, or subject withdrawal.

5.2 Concomitant & Prohibited Therapy Permitted: Standard supportive care for advanced cancer and symptom management.
Prohibited: Systemic immunosuppressive medications (with specific protocol exceptions) and concurrent, unauthorized anti-cancer therapies.

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures	:---
:---	:---	Screening	Day -28 to 0 Informed Consent, Medical History, Baseline Imaging, Labs Baseline Day 0 Enrollment, First Dose, Intensive Pharmacokinetic (PK) tracking
Treatment Period	Treatment Cycles	Safety monitoring, DLT evaluation, PK/ADA sampling, Infusions	End of Study Follow-up RECIST Assessments, Safety Follow-up, Survival tracking

7. Outcome Measures (Endpoints)

7.1 Primary Endpoint Definition: Incidence of adverse events (AEs), Incidence of serious adverse events (SAEs), and Incidence of AEs meeting protocol defined dose-limiting toxicity (DLT) criteria.

Measurement Method: Clinical observation and laboratory assessments graded per standard oncology toxicity criteria (CTCAE).

7.2 Secondary & Exploratory Endpoints * Pharmacokinetics: Maximum observed concentration (C_{max}), Time of maximum observed concentration (T_{max}), and Area under the concentration-time curve (AUC) of BMS-986484.

Immunogenicity: Incidence of anti-drug antibodies (ADAs).

Efficacy: Objective Response Rate (ORR), Disease Control Rate (DCR), and Duration of Response (DOR) measured via RECIST v1.1 imaging.

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Frequent and intensive on-site monitoring, particularly in Cycle 1, to capture AEs and Dose-Limiting Toxicities (DLTs).

Serious Adverse Events (SAEs): Reported standardly within 24 hours to the sponsor pharmacovigilance team.

Data Monitoring Committee (DMC): An internal Safety Review Committee will assess safety data continuously to dictate cohort dose-escalation steps.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: All Treated/Safety Population (participants receiving ≥ 1 dose of study drug); Efficacy Evaluable Population for tumor assessments.

Sample Size Justification: Estimated $N=213$ allows for standard Phase 1 dose-escalation statistical models (e.g., 3+3 or continual reassessment) and sufficient expansion cohorts to characterize the safety profile and preliminary efficacy signals.

Handling of Missing Data: Missing data will not be imputed for safety analyses. Censoring rules will be applied for time-to-event variables in efficacy models.

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki.

Monitoring Plan: High-frequency onsite and remote monitoring due to the First-in-Human nature of the study, ensuring strict

adherence to dose-escalation protocols.

Audit Trail: Utilizing a validated Electronic Data Capture (EDC) system maintaining detailed, timestamped data clarification and edit logs.

11. Study Identifiers & Governance

Primary ID: CA233-0000

Registry Number: NCT06544655

Posting ID: 304306983368310241

Sponsor/Lead Organization: Bristol-Myers Squibb

Study Title: A Study of BMS-986484 Alone and Combination Therapy in Participants With Advanced Solid Tumors

Official Regulatory Title: A Phase 1/1b First-in-human Trial of BMS-986484 as Monotherapy and Combination Therapy in Participants With Advanced Solid Malignancies

12. Clinical Framework (Oncology Specific)

Primary Condition: Advanced Solid Tumors / Advanced Solid Malignancies

Preferred Name: Advanced Malignant Solid Neoplasm

Semantic Type: Neoplastic Process

Condition Definition: A malignant solid neoplasm that has spread extensively to other anatomic sites or is no longer responding to treatment.

Diagnosis Threshold: Histologically/cytologically confirmed advanced, metastatic, or recurrent NSCLC, PDAC, G/GEJC, MSS CRC, or SCCN.

SNOMED ID: 39958004 (Hierarchy: 785317006 | Advanced solid malignant neoplasm | Advanced solid malignant neoplasm (disorder))

Medical Methodology: Phase 1/1b targeted agent and combinatorial oncology therapy.

Optimization Target: Identifying the Maximum Tolerated Dose (MTD) and Recommended Phase 2 Dose (RP2D).

13. Study Procedures & Methodology

Management Pathway: Intensive early-phase oncology trial pathway.

Education Protocol (HCP): Site Initiation Visits (SIV) prioritizing DLT definitions, infusion reaction management, and combination toxicity monitoring.

Education Protocol (Patient): Strict informed consent detailing the robust schedule of blood draws (PKs) and frequent onsite monitoring required.

Quality Audit Mechanism: Real-time data verification by CRAs prior to any dose-escalation committee meetings.

14. Observation & Follow-Up Schedule

Total Duration: Estimated 2-3 years per patient spanning treatment and survival follow-up.

Onsite Visit Cadence: Frequent during early cycles (weekly/bi-weekly for intensive PK and safety profiling), transitioning to standard infusion cycles.

Remote Monitoring Frequency: Inter-cycle assessments tailored to AE progression.

Checklist Review Interval: Regular imaging intervals for RECIST v1.1 tumor assessment throughout the treatment period.

15. Sign-off & Approval

Role	Name	Signature	Date		:---	:---	:---	:---
Principal Investigator	[Insert Name]	_____	[DD-MMM-YYYY]					
Clinical Operations Lead	[Insert Name]	_____	[DD-MMM-YYYY]					
Lead Statistician	[Insert Name]	_____	[DD-MMM-YYYY]					